A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

20 May 2026

Dr Alastair Stark

Dear Dr Stark,

Re: Dr Alison Lilley
DOB: 09/10/1958 Phone: 0400 526 319
Address: 18 Kiing Street, GLEN IRIS, VIC, 3146
Email: lilleya845@gmail.com

Summary

- **For gastroscopy and colonoscopy at St Vincent's Hospital to evaluate worsening dyspepsia and bowel habit changes.**
- **For comprehensive immunological screening and pelvic floor physiotherapy referral.**
- **No action required by you.**

I had the pleasure of conducting appointment with Alison Lilley. Thank you for referring her.

Symptoms and history of presenting complaint

Alison describes a complex constellation of symptoms spanning the last five years, which she dates back to her first Moderna vaccination. Initial symptoms included limb swelling, brain fog, and chest pain. Since then, she has experienced intermittent diarrhoea and constipation, severe facial and body flushing, facial swelling, and wheezing. These episodes occur at any time of day and are occasionally triggered by stress or exertion. She also notes poor skin healing, with small inflamed capillaries on her arms progressing to ulcers that take months to heal.

In the last 12 months, her gastrointestinal symptoms have accelerated. She initially experienced a six-week period of explosive diarrhoea without a clear trigger. This has since transitioned into a state of "stasis" or severe constipation. She currently requires twice-daily Coloxyl and Senokot, plus Movicol, to achieve a bowel movement approximately every three days. She describes a lack of sensation to defaecate and suffers from passive faecal incontinence, which has worsened recently despite being present since the birth of her first child 32 years ago.

Upper gastrointestinal symptoms are also prominent. Alison reports weekly regurgitation and daily heartburn. She recently woke with gastric fluid on her pillow followed by a prolonged coughing fit, she is concerned about nocturnal aspiration. She experiences retrosternal chest pain when her bowels are sluggish. She is currently taking 40 mg of esomeprazole mane, though she was previously taking it without regard to food timing, which likely limited its efficacy.

Systemically, Alison reports profound fatigue, muscle weakness, and frequent migraines (2–3 per week). She has symptoms of rhinitis, red painful eyes, and a dry mouth. She was previously told an ANA was negative five years ago, though she recalls a "p-ANCA" being positive at that time.

Diet

All appointments and enquires

Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Alison maintains a generally healthy diet. She consumes porridge with seeds and grains for breakfast (no milk or sugar) and Greek yoghurt with fruit (blueberries, oranges, strawberries, banana) for lunch. Dinner typically consists of bulk-cooked vegetable-heavy casseroles or bolognese. Her caffeine intake is not excessive, and she does not consume alcohol.

Past medical history and medications

Her medical history is significant for osteoarthritis affecting both large and small joints, a hydatidiform mole requiring chemotherapy, and three episodes of provoked DVT/PE (the last being 32 years ago). She has a history of a right ankle fracture and a left fibula fracture. Notably, she has a history of complex PTSD, the details of which I will not outline but can speak to if necessary a

Current medications include:Esomeprazole 40 mg mane,Fexofenadine 180 mg TDSFamotidineClonidine 150 mcg nocte and 50 mcg mane, Escitalopram (currently tapering, dose 0.8 mg), Oxazepam 1.5 mg PRN, HRT

Treatments trialled before and investigations performed

She has found approximately 50% improvement in flushing symptoms since commencing high-dose fexofenadine and famotidine, for self diagnosed potential mast cell syndrome. She previously trialled aspirin but ceased this due to easy bruising. A recent CT abdomen noted bladder wall thickening, which was followed up with a pelvic ultrasound that was reportedly inconclusive. Psychiatric treatments have included EMDR and various medications, though she is currently successfully tapering her antidepressants.

Social history, family history

Alison is a retired anaesthetist. She currently rents a room and reports feeling geographically and personally safe. She is unfortunately estranged from her two children. Her late husband died of bowel cancer in 2021; left Alison with significant financial debt through fraudulent activity. There is a family history of bowel cancer (husband).

Examination

On examination, she appeared well. Abdominal examination revealed mild tenderness in the epigastrium and right side, but no masses or organomegaly were appreciated. Her arms showed multiple scars from previous ulcerative lesions. Oral examination was unremarkable.

Impression and plan

Alison presents with a complex history, worsening gastro-oesophageal reflux with possible symptoms of oesophageal dysmotility, and constipation with faecal incontinence. While central sensitisation related to her significant trauma history likely plays a role in her visceral hypersensitivity, her age and the progressive nature of her symptoms necessitate formal investigation.

1. I have organised a gastroscopy and colonoscopy at St Vincent's Public Hospital to exclude structural pathology, microscopic colitis, and Barrett's oesophagus.
2. have requested a comprehensive autoimmune screen including ANA, ENA, ANCA, IgE, complement studies, and anticentromere/SCL70 antibodies.
3. I have strongly recommended a review with a pelvic floor physiotherapist to address her incontinence, as her history suggests a non-relaxing pelvic floor.

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4. **I** have advised her to pursue a cystoscopy to investigate the bladder wall thickening noted on CT, particularly given her history of ketamine use.
5. I will review her post her Gastroscopy and colonoscopy.

Ultimately it may be a functional disorder related to her c-PTSD. The timing of the exacerbation of symptoms certainly fits. We did discuss this and how this might be treated with tricyclics, psychotherapy and pelvic floor physiotherapy.

Thank you again for referring Alison.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Alastair Don

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